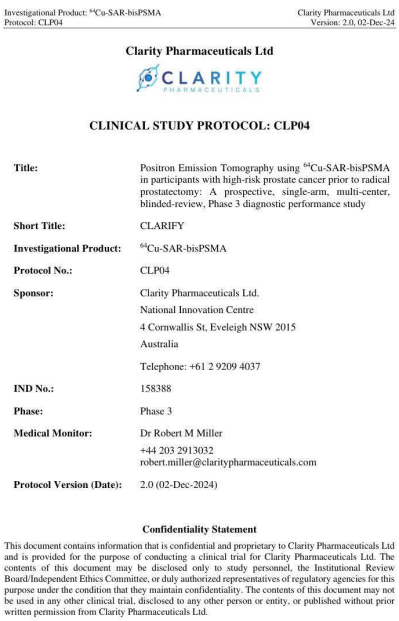
CLARIFY

CLP04 · ^{64}Cu-SAR-bisPSMA PET · Phase 3 diagnostic

P0000 in Board Studio · 80 pages



Investigational Product: ^{64}Cu-SAR-bisPSMA — Clarity Pharmaceuticals Ltd
Protocol: CLP04 — Version: 2.0, 02-Dec-24

Clarity Pharmaceuticals Ltd

CLARITY PHARMACEUTICALS

CLINICAL STUDY PROTOCOL: CLP04

Title:	Positron Emission Tomography using ^{64}Cu-SAR-bisPSMA in participants with high-risk prostate cancer prior to radical prostatectomy: A prospective, single-arm, multi-center, blinded-review, Phase 3 diagnostic performance study
Short Title:	CLARIFY
Investigational Product:	^{64}Cu-SAR-bisPSMA
Protocol No.:	CLP04
Sponsor:	Clarity Pharmaceuticals Ltd. National Innovation Centre 4 Cornwallis St, Eveleigh NSW 2015 Australia Telephone: +61 2 9209 4037
IND No.:	158388
Phase:	Phase 3
Medical Monitor:	Dr Robert M Miller +44 203 2913032 robert.miller@claritypharmaceuticals.com
Protocol Version (Date):	2.0 (02-Dec-2024)

Confidentiality Statement

This document contains information that is confidential and proprietary to Clarity Pharmaceuticals Ltd and is provided for the purpose of conducting a clinical trial for Clarity Pharmaceuticals Ltd. The contents of this document may be disclosed only to study personnel, the Institutional Review Board/Independent Ethics Committee, or duly authorized representatives of regulatory agencies for this purpose under the condition that they maintain confidentiality. The contents of this document may not be used in any other clinical trial, disclosed to any other person or entity, or published without prior written permission from Clarity Pharmaceuticals Ltd.

What this study is, in one breath

One copper-64 PSMA injection in untreated high-risk prostate cancer, two PET time points, then surgery. Pathology of pelvic nodes is the answer key.

Diagnostic PET. Not a therapy. Not FDG.

> **If the protocol feels like a wall of words**
>
> A protocol is a recipe: who can join, what is injected, how much, when you scan or treat, and how you decide if a picture or outcome was right. This sheet is the same idea in normal English. It does not replace the signed site copy.

1. The tracer

Copper-64 SAR-bisPSMA (CLARIFY / CLP04) is a diagnostic PSMA PET agent. Two PSMA-binding arms plus a sarcophagine cage that holds copper-64 so you can image the next day.

- Not FDG. Not a therapy. Not an antibody.
- Cu-64 T½ 12.7 h — long enough for a delayed scan.
- Dose: 200 MBq IV bolus ± 10% of prescribed, once on Day 1.
- No fasting. Hydrate and void; bladder is a critical organ.

2. Who and why

Untreated high-risk prostate cancer, planned for radical prostatectomy plus pelvic lymph-node

dissection. Screen ≤ 28 days. They already have SOC MRI/CT / approved PSMA PET; the Cu-64 scans are extra.

- n = 383. Three central PET readers. Pathologist is blinded to the investigational PET.
- Standard of truth = pelvic-node histopathology, not the SOC PSMA scan.

3. Two PET time points

When	Window	Why
Day 1 PET	1–4 hours after injection, mid-thigh → vertex	Early map; blood pool still visible
Day 2 PET	24 ± 6 hours, same coverage	Clearance → higher tumor-to-background; hunt small nodes
Day 7 ± 4 d	Safety phone call	Then the planned RP + PLND

Co-primary endpoints (sensitivity and specificity vs pathology) are reported independently at each time point. Void immediately before each acquisition.

4. Scoring

PET+ means focal uptake greater than background after excluding physiologic and known benign uptake. TP/FP/FN/TN vs the node the pathologist actually scored. Sensitivity = TP/(TP+FN). Specificity = TN/(TN+FP).

What to know cold

Q. Tracer?
A. Copper-64 SAR-bisPSMA. Two PSMA arms + cage chelator. T½ 12.7 h.

Q. Dose?
A. 200 MBq IV once on Day 1, ±10% of prescribed.

Q. Two scans?
A. Day 1 at 1–4 h and Day 2 at 24 ± 6 h. Co-primaries at each time point.

Q. Who?
A. Untreated high-risk PC heading to RP + PLND. n = 383.

Q. Truth?
A. Pelvic-node histopathology. Pathologist never sees the investigational PET.

How to use this with Board Studio

Open the matching protocol module and press play. The left pane is the signed page. Use this PDF when legalese stalls you. If anything conflicts, the signed protocol wins.

Disclaimer. Educational summary for board-style study. Not medical advice, not a recruitment brochure, not a substitute for the signed protocol, and not affiliated with the sponsor, ABR, ABNM, ACGME, SNMMI, NRC, or FDA. Confirm numbers against the current site copy before any clinical use.